

Leaflet - Eqovex Tablet

Size: 148 x 210mm

Color: Black

Important Information. Please read carefully.

EQOVEX
Film Coated Tablets 90mg & 120mg

COMPOSITION

EQOVEX Film Coated Tablets 90mg: Each film coated tablet contains Etoricoxib 90mg.
EQOVEX Film Coated Tablets 120mg: Each film coated tablet contains Etoricoxib 120mg.

PHARMACODYNAMICS

Pharmacotheapeutic group: Anti-inflammatory and antirheumatic products, non-steroids, coxibs ATC code: M01AH05
Etoricoxib is an oral, selective cyclo-oxygenase-2 (COX-2) inhibitor within the clinical dose range.
Etoricoxib produce dose-dependent inhibition of COX-2 without inhibition of COX-1 at doses up to 150 mg daily. Etoricoxib did not inhibit gastric prostaglandin synthesis and had no effect on platelet function.
Cyclooxygenase is responsible for generation of prostaglandins. Two isoforms, COX-1 and COX-2, have been identified. COX-2 is the isoform of the enzyme that has been shown to be induced by pro-inflammatory stimuli and has been postulated to be primarily responsible for the synthesis of prostanoid mediators of pain, inflammation, and fever. COX-2 is also involved in ovulation, implantation and closure of the ductus arteriosus, regulation of renal function, and central nervous system functions (fever induction, pain perception and cognitive function). It may also play a role in ulcer healing. COX-2 has been identified in tissue around gastric ulcers in man but its relevance to ulcer healing has not been established.

PHARMACOKINETICS

Absorption
Orally administered etoricoxib is well absorbed. The absolute bioavailability is approximately 100%. Following 120 mg once-daily dosing to steady state, the peak plasma concentration (geometric mean C_{max} = 3.6 µg/ml) was observed at approximately 1 hour (T_{max}) after administration to fasted adults. The geometric mean area under the curve (AUC_{0-24hr}) was 37.8 µg•hr/ml. The pharmacokinetics of etoricoxib are linear across the clinical dose range. Dosing with food (a high-fat meal) had no effect on the extent of absorption of etoricoxib after administration of a 120-mg dose. The rate of absorption was affected, resulting in a 36% decrease in C_{max} and an increase in T_{max} by 2 hours.
Distribution
Etoricoxib is approximately 92% bound to human plasma protein over the range of concentrations of 0.05 to 5 µg/ml. The volume of distribution at steady state (V_{ss}) was approximately 120 L in humans.
Metabolism
Etoricoxib is extensively metabolised with <1% of a dose recovered in urine as the parent drug. The major route of metabolism to form the 6'-hydroxymethyl derivative is catalyzed by CYP enzymes. Five metabolites have been identified in man. The principal metabolite is the 6'-carboxylic acid derivative of etoricoxib formed by further oxidation of the 6'-hydroxymethyl derivative. These principal metabolites either demonstrate no measurable activity or are only weakly active as COX-2 inhibitors. None of these metabolites inhibit COX-1.
Elimination
Following administration of a single 25-mg radiolabeled intravenous dose of etoricoxib to healthy subjects, 70% of radioactivity was recovered in urine and 20% in faeces, mostly as metabolites. Less than 2% was recovered as unchanged drug. Elimination of etoricoxib occurs almost exclusively through metabolism followed by renal excretion. Steady state concentrations of etoricoxib are reached within seven days of once daily administration of 120 mg, with an accumulation ratio of approximately 2, corresponding to a half-life of approximately 22 hours. The plasma clearance is estimated to be approximately 50 ml/min.

INDICATIONS

EQOVEX is indicated for:
•Acute and chronic treatment of the signs and symptoms of osteoarthritis (OA) and rheumatoid arthritis (RA)
•Treatment of ankylosing spondylitis (AS)
•Treatment of acute gouty arthritis
•Treatment of acute pain, including that related to primary dysmenorrhoea and minor dental procedures.
The decision to prescribe a selective COX-2 inhibitor should be based on an assessment of the individual patient's overall risks.

DOSEAGE AND ADMINISTRATION

EQOVEX is administered orally and may be taken with or without food. EQOVEX should be administered for the shortest duration possible and the lowest effective daily dose should be used.
Osteoarthritis (OA)
The recommended dose is 30 mg or 60mg once daily.
Rheumatoid arthritis (RA)
The recommended dose is 60 mg once daily. In some patients with insufficient relief from symptoms, an increased dose of 90 mg once daily may increase efficacy. Once the patient is clinically stabilised, down-titration to a 60 mg once daily dose may be appropriate. In the absence of an increase in therapeutic benefit, other therapeutic options should be considered.
Ankylosing spondylitis (AS)
The recommended dose is 60 mg once daily. In some patients with insufficient relief from symptoms, an increased dose of 90 mg once daily may increase efficacy. Once the patient is clinically stabilised, down-titration to a 60 mg once daily dose may be appropriate. In the absence of an increase in therapeutic benefit, other therapeutic options should be considered.
Acute pain
In the following acute painful conditions, EQOVEX should be used only for the acute symptomatic period, limited to a maximum of 8 days treatment:
Acute gouty arthritis
The recommended dose is 120 mg once daily.
Primary dysmenorrhoea
The recommended dose is 120 mg once daily.
Minor dental procedures
The recommended dose is 90 mg once daily.

Doses greater than those recommended for each indication have either not demonstrated additional efficacy or have not been studied. Therefore:
The dose for OA should not exceed 60 mg daily.
The dose for RA and ankylosing spondylitis should not exceed 90 mg daily.
The dose for acute gout should not exceed 120 mg daily.
The dose for acute pain and primary dysmenorrhoea should not exceed 120mg daily.
The dose for minor dental procedures should not exceed 90 mg daily.

Given the association between cardiovascular risk and exposure to COX-2 inhibitors, doctors are advised to use the lowest effective dose for the shortest possible duration of treatment. The patient's need for symptomatic relief and response to therapy should be re-evaluated periodically.

Elderly, Gender, Race

No dosage adjustment is necessary for the elderly or based on gender or race.
Hepatic insufficiency
In patients with mild hepatic insufficiency (Child-Pugh score 5-6), a dose of 60 mg once daily should not be exceeded. In patients with moderate hepatic insufficiency (Child-Pughscore 7-9), the dose should be reduced: a dose of 60mg every other day should not be exceeded, administration of 30mg once daily can also be considered. There are no clinical or pharmacokinetic data in patients with severe hepatic insufficiency (Child-Pugh score >9).
Renal insufficiency
In patients with advanced renal disease (creatinine clearance <30mL/min), treatment with

etoricoxib is not recommended. No dosage adjustment is necessary for patients with lesser degree of renal insufficiency (creatinine clearance ≥30mL/min).

CONTRAINDICATIONS

•Hypersensitivity to the etoricoxib or to any of the ingredients.
•Active peptic ulceration or active gastro-intestinal (GI) bleeding.
•Patients who have experienced bronchospasm, acute rhinitis, nasal polyps, angioneurotic oedema, urticaria, or allergic-type reactions after taking acetylsalicylic acid or NSAIDs including COX-2 (cyclooxygenase-2)inhibitors.
•Pregnancy and lactation.
•Severe hepatic dysfunction (serum albumin <25 g/l or Child-Pugh score ≥10).
•Estimated renal creatinine clearance <30 ml/min.
•Children and adolescents under 16 years of age.
•Inflammatory bowel disease.
•Congestive heart failure (NYHA II-IV).
•Contraindication for etoricoxib in patients with hypertension (high blood pressure) whose blood pressure is not under control.
•Contraindication for patients who have increased risk of cardiovascular disease (ischemic heart disease and stroke).

WARNINGS AND PRECAUTIONS

Gastrointestinal effects
Risk of GI Ulceration, Bleeding and Perforation with NSAID
Serious GI toxicity such as bleeding, ulceration and perforation can occur at any time, with or without warning symptoms, in patients treated with NSAID therapy. Although minor upper GI problems (e.g. dyspepsia) are common, usually developing early in therapy, prescribers should remain alert for ulceration and bleeding in patients treated with NSAIDs even in the absence of previous GI tract symptoms. Studies to date have not identified any subset of patients not at risk of developing peptic ulceration and bleeding. Patients with prior history of serious GI events and other risk factors associated with peptic ulcer disease (e.g. alcoholism, smoking, and corticosteroid therapy) are at increased risk. Elderly or debilitated patients seem to tolerate ulceration or bleeding less than other individuals and account for most spontaneous reports for fatal GI events. There is a further increase in the risk of gastrointestinal adverse effects (gastrointestinal ulceration or other gastrointestinal complications) when etoricoxib is taken concomitantly with acetylsalicylic acid (even at low doses).
Cardiovascular effects
Selective COX-2 inhibitor class of drugs may be associated with a risk of thrombotic events (especially myocardial infarction (MI) and stroke. Given the association between cardiovascular risk and exposure to COX-2 inhibitors, doctors are advised to use the lowest effective dose for the shortest possible duration of treatment. The patient's need for symptomatic relief and response to therapy should be re evaluated periodically, especially in patients with osteoarthritis. Patients with significant risk factors for cardiovascular events (e.g., hypertension, hyperlipidaemia, diabetes mellitus, smoking) should only be treated with etoricoxib after careful consideration. COX-2 selective inhibitors are not a substitute for acetylsalicylic acid for prophylaxis of cardiovascular thrombo-embolic diseases because of their lack of antiplatelet effect. Therefore, antiplatelet therapies should not be discontinued.
Renal effects
Renal prostaglandins may play a compensatory role in the maintenance of renal perfusion. Therefore, under conditions of compromised renal perfusion, administration of etoricoxib may cause a reduction in prostaglandin formation and, secondarily, in renal blood flow, and thereby impair renal function. Patients at greatest risk of this response are those with pre-existing significantly impaired renal function, uncompensated heart failure, or cirrhosis. Monitoring of renal function in such patients should be considered.

Fluid retention, oedema and hypertension
As with other medicinal products known to inhibit prostaglandin synthesis, fluid retention, oedema and hypertension have been observed in patients taking etoricoxib. All Nonsteroidal Anti-inflammatory Drugs (NSAIDs), including etoricoxib, can be associated with new onset or recurrent congestive heart failure. Caution should be exercised in patients with a history of cardiac failure, left ventricular dysfunction, or hypertension and in patients with pre-existing oedema from any other reason. If there is clinical evidence of deterioration in the condition of these patients, appropriate measures including discontinuation of etoricoxib should be taken.
Etoricoxib may be associated with more frequent and severe hypertension than some other NSAIDs and selective COX-2 inhibitors, particularly at high doses. Therefore, hypertension should be controlled before treatment with etoricoxib and special attention should be paid to blood pressure monitoring during treatment with etoricoxib. Blood pressure should be monitored within two weeks after initiation of treatment and periodically thereafter. If blood pressure rises significantly, alternative treatment should be considered.
Hepatic effects
Any patients with symptoms and/or signs suggesting liver dysfunction, or in whom an abnormal liver function test has occurred, should be monitored. If signs of hepatic insufficiency occur, or if persistently abnormal liver function tests (three times the upper limit of normal) are detected, etoricoxib should be discontinued.

General
Warning to prescriber when prescribing COX-2 inhibitors to patients with risk factors of heart disease, hypertension (high blood pressure), hyperlipidemia, diabetes, smoking patient and patient with peripheral arterial disease.
If during treatment, patients deteriorate in any of the organ system functions described above, appropriate measures should be taken and discontinuation of etoricoxib therapy should be considered. Medically appropriate supervision should be maintained when using etoricoxib in the elderly and in patients with renal, hepatic, or cardiac dysfunction. Caution should be used when initiating treatment with etoricoxib in patients with dehydration. It is advisable to rehydrate patients prior to starting therapy with etoricoxib. Serious skin reactions, some of them fatal, including exfoliative dermatitis, Stevens-Johnson syndrome, and toxic epidermal necrolysis, have been reported very rarely in association with the use of NSAIDs and some selective COX-2 inhibitors. Patients appear to be at highest risk for these reactions early in the course of therapy with the onset of the reaction occurring in the majority of cases within the first month of treatment. Serious hypersensitivity reactions (such as anaphylaxis and angioedema) have been reported in patients receiving etoricoxib. Some selective COX-2 inhibitors have been associated with an increased risk of skin reactions in patients with a history of any drug allergy. Etoricoxib should be discontinued at the first appearance of skin rash, mucocut lesions, or any other sign of hypersensitivity.

Etoricoxib may mask fever and other signs of inflammation. Caution should be exercised when co-administering etoricoxib with warfarin or other oral anticoagulants.

The use of etoricoxib, as with any medicinal product known to inhibit cyclooxygenase / prostaglandin synthesis, is not recommended in women attempting to conceive

DRUG INTERACTIONS

Pharmacodynamic interactions
Oral anticoagulants
Concurrent use of etoricoxib and warfarin may result in increased prothrombin time International Normalized Ratio (INR). Patients receiving oral anticoagulants should be closely monitored for their prothrombin time INR, particularly in the first few days when therapy with etoricoxib is initiated or the dose of etoricoxib is changed.
Diuretics, ACE inhibitors and Angiotensin II Antagonists
NSAIDs may reduce the effect of diuretics and other antihypertensive drugs. In some patients with compromised renal function (e.g. dehydrated patients or elderly patients with compromised renal function) the co-administration of an ACE inhibitor or Angiotensin II antagonist and agents that inhibit cyclo-oxygenase may result in further

deterioration of renal function, including possible acute renal failure, which is usually reversible. These interactions should be considered in patients taking etoricoxib concomitantly with ACE inhibitors or angiotensin II antagonists. Therefore, the combination should be administered with caution, especially in the elderly. Patients should be adequately hydrated and consideration should be given to monitoring of renal function after initiation of concomitant therapy, and periodically thereafter.

Acetylsalicylic Acid
Etoricoxib can be used concomitantly with acetylsalicylic acid at doses used for cardiovascular prophylaxis (low-dose acetylsalicylic acid). However, concomitant administration of low-dose acetylsalicylic acid with etoricoxib may result in an increased rate of GI ulceration or other complications compared to use of etoricoxib alone. Concomitant administration of etoricoxib with doses of acetylsalicylic acid above those for cardiovascular prophylaxis or with other NSAIDs is not recommended.
Ciclosporin and tacrolimus
Coadministration of ciclosporin or tacrolimus with any NSAID may increase the nephrotoxic effect of ciclosporin or tacrolimus. Renal function should be monitored when etoricoxib and either of these drugs is used in combination.

Pharmacokinetic Interactions
The effect of etoricoxib on the pharmacokinetics of other drugs
Lithium
NSAIDs decrease lithium renal excretion and therefore increase lithium plasma levels. If necessary, monitor blood lithium closely and adjust the lithium dosage while the combination is being taken and when the NSAID is withdrawn.
Methotrexate
Concurrent use of etoricoxib and methotrexate may result in increased methotrexate plasma concentrations and toxicity. Adequate monitoring for methotrexate-related toxicity is recommended when etoricoxib and methotrexate are administered concomitantly.
Oral contraceptives
Concurrent use of etoricoxib and ethinyl estradiol (Oral contraceptives) may result in increased plasma concentration of ethinyl estradiol. This increase in EE concentration should be considered when selecting an oral contraceptive for use with etoricoxib. An increase in EE exposure can increase the incidence of adverse events associated with oral contraceptives (e.g., venous thrombo-embolic events in women at risk).
Hormone Replacement Therapy (HRT)
Concurrent use of etoricoxib and conjugated estrogens or Hormone Replacement Therapy may result in increased conjugated estrogen exposure. These increases in estrogenic concentration should be taken into consideration when selecting post-menopausal hormone therapy for use with etoricoxib because the increase in oestrogen exposure might increase the risk of adverse events associated with HRT.
Prednisone/prednisolone
Etoricoxib did not have clinically important effects on the pharmacokinetics of prednisone/prednisolone.

Digoxin
Patients at high risk of digoxin toxicity should be monitored for this when etoricoxib and digoxin are administered concomitantly.
Effect of etoricoxib on drugs metabolised by sulfo transferases
Etoricoxib is an inhibitor of human sulfo transferase activity, particularly SULT1E1, and has been shown to increase the serum concentrations of ethinyl estradiol. While knowledge about effects of multiple sulfo transferases is presently limited and the clinical consequences for many drugs are still being examined, it may be prudent to exercise care when administering etoricoxib concurrently with other drugs primarily metabolised by human sulfo transferases (e.g., oral salbutamol and minoxidil).
Effect of etoricoxib on drugs metabolised by CYP isoenzymes
Etoricoxib is not expected to inhibit cytochromes P450 (CYP) 1A2, 2C9, 2C19, 2D6, 2E1 or 3A4.
Effects of other drugs on the pharmacokinetics of etoricoxib
The main pathway of etoricoxib metabolism is dependent on CYP enzymes. CYP3A4 appears to contribute to the metabolism of etoricoxib in vivo. CYP2D6, CYP2C9, CYP1A2 and CYP2C19 also can catalyse the main metabolic pathway, but their quantitative roles have not been studied.

Ketoconazole
Ketoconazole, a potent inhibitor of CYP3A4 did not have any clinically important effect on the single-dose pharmacokinetics of 60 mg etoricoxib.
Voriconazole and Miconazole
Co-administration of either oral voriconazole or topical miconazole oral gel, strong CYP3A4 inhibitors, with etoricoxib caused a slight increase in exposure to etoricoxib, but is not considered to be clinically meaningful based on published data.
Rifampicin
Co-administration of etoricoxib with rifampicin, a potent inducer of CYP enzymes, produced a decrease in etoricoxib plasma concentrations. This interaction may result in recurrence of symptoms when etoricoxib is co-administered with rifampicin. While this information may suggest an increase in dose, doses of etoricoxib greater than those listed for each indication are not recommended.
Breast-feeding
Etoricoxib is excreted in the milk of lactating rats It is not known whether etoricoxib is excreted in human milk. Women who use etoricoxib must not breast-feed.

EFFECT ON ABILITY TO DRIVE AND USE MACHINES
Patients who experience dizziness, vertigo or somnolence while taking etoricoxib should refrain from driving or operating machinery.

SIDE EFFECTS

Tabulated list of adverse reactions

System Organ Class	Adverse Reactions	Frequency Category
Infections and infestations	Alveolar osteitis	Common
	Gastroenteritis, upper respiratory infection, urinary tract infection	Uncommon
Blood and lymphatic system disorders	Anaemia (primarily associated with gastrointestinal bleeding), leukopenia, thrombocytopenia	Uncommon
Immune system disorders	Hypersensitivity	Uncommon
	Angioedema/anaphylactic/ anaphylactoid reactions including shock	Rare

KLF0108N1

Metabolism and nutrition disorders	Oedema/fluid retention	Common
	Appetite increase or decrease, weight gain	Uncommon
Psychiatric disorders	Anxiety, depression, mental acuity decreased, hallucinations	Uncommon
	Confusion, restlessness	Rare
Nervous system disorders	Dizziness, headache	Common
	Dysgeusia, insomnia, paresthaesia/ hypaesthesia, somnolence	Uncommon
Eye disorders	Blurred vision, conjunctivitis	Uncommon
Ear and labyrinth disorders	Tinnitus, vertigo	Uncommon
Cardiac disorders	Palpitations, arrhythmia	Common
	Atrial fibrillation, tachycardia, congestive heart failure, non-specific ECG changes, angina pectoris, myocardial infarction	Uncommon
Vascular disorders	Hypertension	Common
	Flushing, cerebrovascular accident, transient ischaemic attack, hypertensive crisis, vasculitis	Uncommon
Respiratory, thoracic and mediastinal disorders	Bronchospasm	Common
	Cough, dyspnoea, epistaxis	Uncommon
Gastrointestinal disorders	Abdominal pain	Very common
	Constipation, flatulence, gastritis, heartburn/acid reflux, diarrhoea, dyspepsia/ epigastric discomfort, nausea, vomiting, oesophagitis, oral ulcer	Common
	Abdominal distention, bowel movement pattern change, dry mouth, gastroduodenal ulcer, peptic ulcers including gastrointestinal perforation and bleeding, irritable bowel syndrome, pancreatitis	Uncommon
	ALT increased, AST increased	Common
Hepatobiliary disorders	Hepatitis	Rare
	Hepatic failure, jaundice	Rare
Skin and subcutaneous tissue disorders	Echymosis	Common
	Facial oedema, pruritus, rash, erythema, urticaria	Uncommon
Musculoskeletal and connective tissue disorders	Stevens-Johnson syndrome, toxic epidermal necrolysis, fixed drug eruption	Rare
	Muscular cramp / spasm, musculoskeletal pain/stiffness	Common
Renal and urinary disorders	Proteinuria, serum creatinine increased, renal failure/renal insufficiency	Uncommon
General disorders and administration site conditions	Asthenia/fatigue, flu-like disease	Common
	Chest pain	Uncommon
Investigations	Blood urea nitrogen increased, creatine phosphokinase increased, hyperkalaemia, uric acid increased	Common
	Blood sodium decreased	Uncommon

The following serious undesirable effects have been reported in association with the use of NSAIDs and cannot be ruled out for etoricoxib: nephrotoxicity including interstitial nephritis and nephrotic syndrome.

SYMPTOMS AND TREATMENT FOR OVERDOSAGE

In clinical studies, administration of single doses of etoricoxib up to 500 mg and multiple doses up to 150 mg/day for 21 days did not result in significant toxicity. The most frequently observed adverse experiences were consistent with the safety profile for etoricoxib (e.g. gastrointestinal events, cardiorenal events). In the event of overdose, it is reasonable to employ the usual supportive measures, e.g., remove unabsorbed material from the GI tract, employ clinical monitoring, and institute supportive therapy, if required. Etoricoxib is not dialysable by haemodialysis; it is not known whether etoricoxib is dialysable by peritoneal dialysis.

STORAGE CONDITION

Store below 30°C.

SHELF LIFE

The expiry date is indicated on the packaging.

PRODUCT DESCRIPTION,DOSEAGE FORM AND PACKAGING AVAILABLE

EQOVEX Film Coated Tablets 90mg: White, round, normal convex tablet with "XS" marking on one side and plain on the other side.
Available in pack sizes of 3x10 tablets.

EQOVEX Film Coated Tablets 120mg: Green, round, normal convex tablet with "XS" marking on one side and plain on the other side.
Available in pack sizes of 3x10 tablets.

For further information, please consult your physician or pharmacist.

Revision Date: 19-11-2024



Manufacturer and Product Registration Holder:
Xepa-Soul Pattinson (Malaysia) Sdn Bhd
1-5 Cheng Industrial Estate, 75250 Melaka, Malaysia.